



During transition, you may be overwhelmed by the strength and frequency of your contractions. Your partner's support is crucial during this stage and their encouragement can help keep you focused on your labor.

FAILURE TO PROGRESS IN LABOR

If your cervix is not dilating, or your baby is not descending, as quickly as expected during the first stage, your doctor will try to assess why this is and if something can be done. Usually, your doctor will assess the three Ps: the passenger (the size of the baby and his position in the uterus); the powers (the efficiency of your contractions); and the passage (the size and shape of your pelvis). These three elements work together and each one is important for your labor to progress smoothly.

There are several reasons a labor may not progress. These include if the baby's head is too large for the mother's pelvis, known as cephalopelvic disproportion (CPD); if contractions are infrequent or not strong enough; and if the baby is in a posterior position with their back facing the mother's back.

Cephalopelvic disproportion Sometimes CPD may be suspected before labor, in late pregnancy. This may be the case if the doctor thinks that you have a narrow pelvis or prominent sacral bone, both of which may make birth slower or more difficult. However, an assessment of the pelvis alone is not an accurate way to predict if you'll be able to have a successful vaginal birth and, even if the pelvis is not an optimal shape, the doctor may be happy for you to continue trying for a vaginal birth. This is because it's not the shape of your pelvis alone that is important, but the interaction between your baby (the passenger) and your pelvis.